

Juno Open Health Tools

What gets lost before the appointment?

A co-authored data brief on communication barriers and low-burden preparation for people with ME/CFS

Proposed by: Juno / Juno Open Health Tools	Proposed collaborator: Action for M.E. and the PRIME research involvement community
Status: Scoping proposal - no data collection has begun	Suggested first step: A 30-minute fit and governance conversation

The proposal. Co-produce a short, public data brief on what people with ME/CFS find hardest to communicate in time-limited appointments, which preparation methods are genuinely usable, and what patients want clinicians to understand before the conversation begins.

Why this is worth testing

Fluctuating symptoms, post-exertional worsening, cognitive load and a changing baseline can be difficult to compress into a few minutes. The result is not simply an information problem: people may leave without having explained the change that mattered most or asked the question they came to ask.

Action for M.E. places lived experience at the centre of its services and research partnerships. Juno has built an openly licensed, non-diagnostic appointment-preparation method from lived experience of chronic illness. A tightly scoped collaboration could turn those complementary strengths into useful evidence and practical resources without positioning either organisation as a clinical authority.

Proposed research question

What information do people with ME/CFS most struggle to communicate before and during appointments, which low-burden preparation supports help them express changes and priorities more clearly, and what do they want clinicians to ask or acknowledge?

Communication. Which experiences are hardest to describe, prioritise or put on a short timeline?

Usability. Which preparation formats remain manageable with fatigue, pain, sensory sensitivity or cognitive dysfunction?

Equity. What adjustments are needed for people with severe ME, carers and people whose access needs are often excluded from standard digital tools?

A small, governance-first study

The design should be co-produced and intentionally low burden. Before any recruitment, the partners would decide whether the work is patient and public involvement, service evaluation or research, identify the appropriate ethics and data-protection route, and agree who is the data controller.

Phase	Indicative timing	Decision or output
1. Scope	Week 1	Agree the question, classification, governance, access needs and stop/go criteria.
2. Co-design	Week 2	A small lived-experience group reviews language, burden, accessibility and response options.
3. Listen	Weeks 3-4	If approved, run a short anonymous survey and optional low-burden conversations through agreed channels.
4. Interpret	Week 5	Review aggregate findings with lived-experience contributors; explicitly separate observation from inference.
5. Publish	Week 6	Release the co-authored brief, practical conversation guide, methods note and limitations.

Safeguards and ownership

No patient records. The project would not request clinical documents, account credentials, record numbers or information copied from a clinical system.

Minimum necessary data. Collect only what is needed for the agreed question; avoid direct identifiers and free-text prompts that invite unnecessary disclosure.

No commercial lead generation. Participation would not be used to market Juno, build sales lists or target people with advertising.

Shared interpretation and veto. Action for M.E. and lived-experience contributors would help interpret findings and approve co-branding; both partners could stop publication if safety or accuracy concerns remain.

Proposed public outputs

Data brief. A concise 4-6 page narrative reporting aggregate themes, response distributions, methods and limitations.

Conversation guide. A one-page resource for patients and clinicians focused on change from baseline, impact, timeline and priorities.

Methods appendix. Question wording, accessibility adaptations, classification decisions and a transparent account of what the study cannot conclude.

Open practical template. An adapted appointment-preparation sheet released under an agreed licence, with no participant-level data published.

What each partner could contribute

Action for M.E. / PRIME. Lived-experience governance, research/PPI classification, access and safeguarding advice, contributor relationships, interpretation and publication approval.

Juno. Initial drafting, open templates, accessible prototyping, analysis support under the agreed governance structure, design and publication production.

The immediate ask. Would Action for M.E. be open to a 30-minute scoping call to decide whether this question is useful, who should shape it, and what governance route would be proportionate? We are comfortable starting with a very small pilot or deciding together that a different question would be more valuable.

Background and open materials

Action for M.E. PRIME research involvement programme: [Open programme page](#)

Juno Open Health Tools - permanent Zenodo archive: [Open archive](#)

Juno Open Health Tools - source repository: [Open repository](#)

Plain-language guide to the appointment-preparation method: [Read the guide](#)

This concept note is for discussion only. It is not a research protocol, recruitment notice, medical advice or claim of clinical effectiveness. No participant data has been collected.